

Monitoring Manager – Assignment

A. Noora Health currently implements the CCP across 658 health facilities in 68 districts across all 8 divisions of Bangladesh. The two attached files describe this same underlying program but come from different reporting layers and time windows, and are not perfectly reconciled with each other.

- CCP_Monitoring_Data: district-level snapshot (68 districts): sessions by condition area, patients, caregivers, trainers, % Sessions vs Target, and CCP Quality Assessment Coverage %.
- Average_sessions_details: facility-level historical data (658 facilities, across 8 facility categories from Medical University down to Union Health & Family Welfare Centres): sessions and participants since each facility went live, with monthly averages.

Task:

1. Using 'CCP_Monitoring_Data', roll up the district-level figures to division level and calculate a division-level "% Sessions vs Target" (weighted by session volume, not a simple average of the district percentages). Rank the 8 divisions from strongest to weakest.
2. Pick any 5 districts. For each, compare the district's total sessions this quarter (CCP_Monitoring_Data file) against what you'd expect for a quarter based on that district's facilities in Average_sessions_details file (their average monthly sessions $\times$ 3). Which districts, if any, look inconsistent between the two sources and for those, what's your hypothesis for the gap? For the rest, do you expect the figures from the two sources to be similar, or would you want to verify the figures?
3. Recommend the 3 divisions you would prioritize for management attention this quarter. State your indicators, it should combine more than one data dimension, and describe, in words, the dashboard you'd design for the Implementation Team using this kind of data. For each visual you propose, name the one decision or question it should help answer.
4. Write a 200-250 word executive summary of your findings addressed to the Country Director, who has 5 minutes to read it before a leadership meeting.

- B. Noora Health is planning to expand the CCP to 15 new upazila (sub-district) hospitals located in char (river-island) and haor (wetland) areas. These facilities are 4–6 hours' travel one-way from the nearest divisional office, have unreliable mobile internet, and have historically had high nurse turnover (trained nurses often transfer out within 6–9 months).

Task:

1. Propose a monitoring approach for these 15 facilities given the connectivity and travel constraints described. Specify: (a) which 4–5 indicators you would prioritize tracking and why, (b) how data would be collected given intermittent connectivity, and (c) a realistic field-visit frequency, with your reasoning for why that frequency is achievable and sufficient. Identify the top 3 risks specific to this expansion and your mitigation approach for each.
 2. One monitoring officer on your team will be responsible for covering 5 of these facilities largely on their own. What would you personally put in place, as their manager, to support them and protect data quality, without requiring them to travel more than is realistic? Answer in 4–6 sentences.
- C. Below is a draft monitoring plan for the Noora Health program launch. It has several weaknesses.

Draft: "We will track the number of sessions conducted and number of participants, self-reported monthly by each facility's lead nurse via a WhatsApp message to the Program Managers (PMs). The Monitoring Team will compile this into a single master spreadsheet, shared with the research team who will also use it for their impact study. We will review the numbers once a quarter with the Implementation Team and flag any facility that reports zero sessions."

Task:

1. Identify at least 3 specific weaknesses in this draft plan, and for each, explain the concrete risk it creates for Noora Health. Then rewrite the draft to fix it, in 3–5 sentences.
- D. Below are anonymized notes collected by monitoring and implementation staff during recent facility visits and monthly calls, the kind of unstructured input a Monitoring Manager is expected to make sense of and turn into program learning.
- *"The flipchart images are a bit outdated; new mothers keep asking why the illustration shows a hospital bed different from ours. It's a small thing, but it throws them off."*
 - *"Three of our best-trained nurses at Jashore transferred in the last two months. The new ones need more supervision, but I don't have the bandwidth to visit that often."*
 - *"Mothers seem more engaged when the sessions happen right after their morning vitals check, rather than in the evening slot. Attendance drops noticeably in the evening sessions."*
 - *"I had to fill out the SurveyCTO form twice this week because the app crashed halfway through and didn't save anything. I've started keeping a paper backup now."*
 - *"During Ramadan, session timing needs to shift; we had almost no attendance in the 1 pm slot for two weeks straight."*
 - *"One of the fathers waiting outside asked if he could join the session; we said no because it's meant for mothers and female caregivers, but he seemed really eager to learn."*
 - *"The PM visited last month and reviewed our numbers but didn't ask us about the connectivity issues we've been having; we've mentioned it three times now in our monthly calls."*
 - *"A caregiver told us she'd already learned the same information from a poster in the hallway, so she left the session early. We might be duplicating some content."*

Task:

1. Group the 8 notes above into 3-4 themes of your own choosing. For each theme: give it a short label, write one sentence on the underlying pattern it points to, not a summary of the quotes, but what it tells us about the program, and say whether you'd treat it as an isolated incident or something to escalate as systemic, with your reasoning.
2. Draft a 100-150 word Program Learning Note, to share qualitative insight with the design and implementation teams based on your synthesis above. Propose one concrete process or tool change you'd recommend so this kind of feedback gets captured more systematically going forward.

Format:

- PDF format
- Cite references if used

Deadline:

Within 72 hours of receiving this assignment (by 11:59 PM)

If you have any questions, feel free to reply to the email thread or reach out directly to shourav@noorahealth.org.

Good luck!